

ONTARIO

SUPERIOR COURT OF JUSTICE

B E T W E E N:

DEANNA ST.PRIX-ALEXANDER,
MICHAEL ALEXANDER AND SKYLER
ALEXANDER, GABRIELLE ALEXANDER
and NATHANIEL ALEXANDER by their
LITIGATION GUARDIAN, DEANNA
ST.PRIX-ALEXANDER

Plaintiffs

- and -

HOME DEPOT OF CANADA INC.

Defendant

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)
) Gregory P. Kelly, Q.C./ Caroline C. Failes,
) for the Plaintiffs
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) Paul Muirhead, for the Defendant
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) **HEARD:** April 10, 11, 12, 13, 16, 17, 18,
20, 23, 25, 2007

MANTON J.

JUDGMENT

[1] On November 27, 1999, Deanna St.Prix-Alexander (the “Plaintiff” or “Deanna”) attended at the store owned and operated by the Defendant Home Depot of Canada Inc. (“Home Depot”)

located at 1616 Cyrville Road in the City of Ottawa with her husband, Michael Alexander and their children.

[2] Deanna went to the kitchen and bath department to look at the toilet displays. She was carrying one of her children in a bunting bag strapped to her chest while her husband Michael went somewhere else in the store to look for other items with the two other children. While Deanna stood in the aisle looking at the display of toilets shown to her by one of the clerks, she was struck from behind by a sharp blow to her neck and shoulder area causing her to see stars. She saw black and white and believes she lost consciousness for a few seconds and she felt immediate pain in her neck, shoulder and head. She recalls screaming and hearing her baby cry and someone bringing her a chair to sit down and giving her a glass of water. She was able to tell someone that she was with her husband and described him so that he could be found in the store. When her husband appeared, he asked her what happened and was told by an employee of Home Depot that his wife was hit on the head with a box.

[3] According to the “customer incident statement” prepared by the manager of Home Depot, Benoît Laliberté, one Mark Reid, an employee, was retrieving a pedestal sink for a customer pulling it off the shelf when he turned around and struck the Plaintiff on the head and neck. When describing the nature of the injury, the manager wrote in the said incident statement that the Plaintiff had head and neck pain and she was instructed to seek medical attention.

[4] Two former employees of Home Depot testified for the defense. Mark Reid, referred to in the “customer incident statement”, worked for Home Depot for approximately two years. He is

the one who pulled the box off the shelf and struck the Plaintiff but he does not recall the incident and could not tell us anything about it.

[5] The second witness called by the defense was Martin Lauzon. At the time of the incident, he had also been employed by Home Depot for approximately two years. He recalls the incident and said that Mark Reid was bringing down a box from a shelf when he nudged or struck the Plaintiff with his elbow or the box. He said that the box in question contained a pedestal sink that was on a shelf at about Mark Reid's eye level before he took it down from the shelf to the floor. Mr. Lauzon asked the Plaintiff if she was okay and he got a chair and helped her sit down on it and then he called the manager, Mr. Laliberté. Mr. Laliberté came to the scene, spoke to the Plaintiff and Mr. Lauzon left at that time because he said that he was no longer needed.

[6] Corey Kennedy, the Manager of the Home Depot on Cyrville Road since 2003, was also called as a witness. He informed us that the number of employees at the Home Depot on Cyrville Road varies from 230 to 280 depending on the time of year. He produced a book entitled "Home Depot of Canada Document Brief" which is used by Home Depot to train new employees. He stated that employees hired by the store had to attend a two day orientation session dealing with such subjects as safe work practice, loss prevention and ethics in dealing with the customers but he could not tell us that Mark Reid had attended these sessions and that direction was provided to Mr. Reid as to the safe retrieval of products from shelves because as he said, he was not employed by the store until 2003.

[7] In cross examination, Mr. Kennedy said he had only heard of this accident last week and that he knew nothing about it. He admitted that the former manager Benoît Laliberté still resided

in the city of Ottawa. It was not explained to the court why Mr. Laliberté was not called as a witness. He was present on the day of the accident, he wrote the customer incident statement and he would know a lot more about the incident than the present manager of the store. No explanation having been given to the court preventing the production of Mr. Laliberté as a witness, and since a *prima facie* case has been made out against the Defendant, I find that it is apparent that the evidence of Mr. Laliberté would not have been favourable to the Defendant (*Bernardi v. Guardian Royal Exchange Assurance Company*, [1979] O.J. No. 553 (Ontario C.A.)).

[8] It is obvious from the evidence of the Plaintiff and Mr. Martin that Mr. Reid was negligent in performing his duties. He was retrieving a box containing a pedestal sink from a shelf at his eye level and was negligent in that he did not or could not handle it and bring it safely to the floor. Mark Reid should have seen the Plaintiff in the aisle looking at the display of toilets and should have waited until she was in a safe place before removing the box. He failed to keep a proper lookout for customers in the aisle when removing the box from the shelf and placed the Plaintiff in a situation of danger.

[9] The *Occupiers' Liability Act*, R.S.O. 1990, c.O. 2 imposes a duty upon the occupier to take reasonable care for the safety of people who are permitted on the premises. Mark Reid did not take this reasonable care for the safety of the Plaintiff who was an innocent by-stander in the aisle looking at goods displayed in the Defendant's store. I therefore find that the injuries sustained to the Plaintiff at Home Depot and resulting damages arose out of the sole negligence

of Home Depot and their employee, Mark Reid, for whose negligence, the Defendant is vicariously liable.

[10] The Plaintiff Deanna St.Prix-Alexander states that she was seriously injured and suffered substantial damages as the result of this accident and claims general damages, loss of income and competitive advantages (amended on consent at trial to \$4,000,000) and special damages being her past loss of income and OHIP's subrogated loss. The claim for punitive damages was withdrawn at trial. The Plaintiff's husband and children claim damages pursuant to the *Family Law Act*, R.S.O. 1990, c.F. 3.

[11] The Plaintiff attended a medical clinic about one hour after the incident and indicated to the doctor that a box hit her at the back of the head and neck and she complained about pain at the back of the neck, the left shoulder, the upper body and the chest. The doctor observed that she was in obvious stress and she was tender over the C-5 and C-6 and on the left shoulder. On December 2, 1999, she saw her family doctor, Dr. Morris, and told him that she continued to have a lot of pain especially in the back of her head and on the left side and that she could not lift. She also complained of neck stiffness since the time of the injury. Dr. Morris sent her for an X-ray of the cervical spine. The X-ray report of the same date shows that at the time, the Plaintiff had a normal neck apart from a straightening of the lordosis. Because the pain was persisting, Dr. Morris recommended physiotherapy. She attended three sessions of physiotherapy in December 1999. After two sessions at the physiotherapy clinic, the physiotherapist, Janet Sprague, wrote to Dr. Morris indicating that her subjective findings were that the Plaintiff had pain in the neck and in the left shoulder and felt pins and needles in her left hand. On

December 16, 1999, the Plaintiff reattended at Dr. Morris with the same complaints and he recommended that she continue her physiotherapy, apply heat and rest as much as possible. The Plaintiff did not return to physiotherapy until March 30, 2000 and attended five sessions until April 26, 2000 and five more sessions of physiotherapy from June 5 to June 26, 2000. During that time, she was treated for lower neck and upper thoracic pain and stiffness, thoracic joint restriction and stiffness in her shoulders. The physiotherapist said that she had developed bilateral shoulder tendonitis as a result of the restrictions in her thoracic spine that had not fully resolved.

[12] The Plaintiff was then referred to Dr. Lynne MacGregor, a doctor specializing in physical medicine and rehabilitation. She was under Dr. MacGregor's care from September 2000 to September 2001. This doctor's clinical impression was that the Plaintiff suffered from myofascial neck pain and tension headaches. She recommended that the Plaintiff double her medication for one month, decrease her workload and told her that she may benefit from going to the pain clinic.

[13] While under Dr. MacGregor's care, the Plaintiff attended eight physiotherapy sessions from October 20, 2000 to January 2001 but these, she said, did not help her very much.

[14] Dr. MacGregor recommended that she consult Dr. Jackson, a chiropractor. From November 2001 until January 2002, she attended for treatment by him on ten occasions. On her first visit November 16, 2001, the Plaintiff described her current problems as pain in the shoulder, neck, lack of body motion, and headaches causing lack of ability to work. She stopped seeing him because she found his treatment was too painful.

[15] As time passed, the Plaintiff did not see improvements in her condition, her neck was still sore and she felt as if she was getting worse instead of better. She began to worry about not being strong enough to continue with her employment. She wanted to progress in her work and enjoy life. She was wondering what the future held for her and worried because she did not have the strength to participate in activities with her children and go on with an ordinary life.

[16] On November 1, 2001, she discontinued working on a full time basis and worked four days per week instead because her physical condition just deteriorated.

[17] Dr. Morris referred the Plaintiff to Dr. R. Lenz, a psychiatrist. She was seen by Dr. Lenz on January 31st and March 22, 2002. At that time, Dr. Lenz reported to Dr. Morris that the Plaintiff stated to him that her main issues were her marital problems, her neck pain and her career issue. Dr. Lenz recommended that she consider marital therapy and also consider a referral to a chronic pain clinic. She did follow-up with a few marital therapy sessions and she also changed job to something less stressful which will allow her more flexibility. Dr. Lenz recommended that she continue with the marital therapy because he said this will likely take several months if not years to come to some resolution and recommended to Dr. Morris that he send the Plaintiff for a referral to the chronic pain clinic for further assessment.

[18] During the month of December 2002, the Plaintiff became very upset with Dr. Morris, her family doctor. She was wondering whether he took her complaints seriously. She felt that there was something very wrong with her body. She had burning and tingling and cold sensation in her arm, leg and the left side of her body. She was so upset that she swore at Dr. Morris and insisted that he prescribe an MRI. Dr. Morris then referred the Plaintiff to Dr. Eric Bélanger, a

neurosurgeon. During the month of January 2003, an MRI was obtained and showed that the Plaintiff had ossification of her posterior longitudinal ligament (“OPLL”) with cervical spinal stenosis which was worse at C3, C4 and then moderate at C4, C5 and at C5-C6. After viewing the MRI, it was recommended by Dr. Bélanger that the Plaintiff have a decompression procedure on the cervical spine. She was told by him that this was a very serious operation and if anything went wrong, she could be paralyzed or she could die as a result. Naturally, she was very worried; especially that Dr. Bélanger had told her that the operation might not relieve her neck pain. Dr. Bélanger told her that the main goal of the surgery was to prevent neurological deterioration and hopefully improve her left-sided cervical radiculopathy.

[19] As a result of the information received from Dr. Bélanger, she went back to Dr. Morris and he referred to Dr. Ivan Woods, a neurologist at the McGill University Health Center for a second opinion. Dr. Woods studied the MRI and examined the Plaintiff on February 27, 2003 and he came to the same conclusion as Dr. Bélanger and referred her to Dr. Denis J. Sirhan, a neurosurgeon at the same health center. This neurosurgeon explained to her that the goals of the surgery are mainly preventive but he believed that she would regain some strength and some sensation in both upper extremities and even some parts of her thorax. He concluded that she would be put on an urgent list if she decided to have surgery in Montréal but the Plaintiff decided to be operated in Ottawa by Dr. Bélanger. The surgical intervention was performed by Dr. Bélanger on April 30, 2003 to correct severe stenosis at C3 and C4 and moderated stenosis at C4, C5 and C5-C6. She underwent an interior decompression associated with reconstruction. Following the operation, she was hospitalized for a week. During that week, she could not swallow, eat or drink. She said that she felt a lot of pain, had difficulty moving her body and

could not walk for a period of two weeks following her discharge from the hospital. She had difficulty taking a shower or combing her hair. She had to be helped for almost every movement she made for about two months and had to depend on her husband to help her do almost everything. She had to wear a neck brace almost constantly for a period of five months after the operation. She wears this neck brace intermittently, even did so during the trial. She returned to her employment for a few hours per day commencing in August or September 2003.

[20] Dr. Bélanger said in a letter dated September 2003 that the Plaintiff has recovered well from the cervical surgery nevertheless, she is left with residual numbness. He adds that because of the extent of her surgery, she is at risk of requiring further cervical surgery in the future. He concludes that because of the significant cervical spinal cord injury, it is likely that she will remain with long term *sequelae* such as numbness, tingling and headaches amongst others.

[21] The Plaintiff was referred to Dr. Stotts, a neurologist, by Dr. Bélanger for evaluation of headaches following the April 2003 surgery. Dr. Stotts first saw the Plaintiff on March 4, 2004. He describes the Plaintiff's symptoms as remaining unchanged since the 2003 surgery. He said the Plaintiff experienced severe headaches originating with neck pain about every three weeks. The headaches could be triggered and aggravated by the Plaintiff's sensitivity to light, loud noises and certain smells. The headaches were associated with a mild nausea that could last for several days. He told us that the Plaintiff's neck can be exacerbated by sitting.

[22] Dr. Stotts confirmed that there were no obvious symptoms of neurological deficit associated with the Plaintiff's headaches as of March 4, 2004. A remarkable feature noted by Dr. Stotts in his clinical evaluation was that the Plaintiff was extremely tender to touches around

her neck, even light touches to the muscle or skin. His clinical examinations reveal that the Plaintiff's entire left arm showed diminished responses to light pin pricks.

[23] In March 2004, Dr. Stotts said he found no neurological signs that would explain the Plaintiff's neck pain, headaches, and other symptoms.

[24] Dr. Stotts explains the presence of neurological signs exhibited by the Plaintiff's pre-surgery as compared to the lack of neurological signs post-surgery. He says that the reason is that the surgery took pressure off the nerve structures.

[25] Dr. Stotts believes that the Plaintiff's headaches are cervicogenic in nature, meaning that the headaches are generated by the tissue structures supporting the head. It is his opinion that the surgery was unlikely to have affected the Plaintiff's headaches or neck pain but that it was necessary to avoid subsequent spinal cord compression. Dr. Stotts found it harder to explain the Plaintiff's paresthesias following the 2003 surgery however he believes that the symptoms of paresthesias, numbness, tingling and cold as well as her difficulty in feeling light touch and pin pricks on her left arm are due to the 1999 accident at Home Depot.

[26] Dr. Stotts concludes that the 1999 incident was causative of the headaches he was treating the Plaintiff for as she did not have frequent substantial and long lasting headaches before the accident.

[27] On December 31, 2004, the Plaintiff slipped on ice and hit the back of her head. As a result of this, Dr. Stotts re-examined her on January 11, 2005 but he did not change his diagnosis.

[28] The conclusion reached by Dr. Stotts is that it is unlikely that the Plaintiff's chronic pain will substantially diminish given its lengthy history.

[29] Dr. Charles Tator saw the Plaintiff for a medical legal assessment and examined her on November 25, 2005. Dr. Tator received his fellowship in neurosurgery in 1999. He has to his credit 200 publications and 685 papers including one paper on thoracic outlet syndrome. He has performed several thousands of spinal cord surgeries and he has received the Order of Canada medal for his achievement in medicine.

[30] Dr. Tator's opinion after examining the Plaintiff, reading the reports and notes of the several doctors who treated the Plaintiff previously and reviewing the X-ray prescribed by Dr. Morris obtained at the request of Dr. Bélanger was that the Plaintiff sustained an injury to the cervical spine which produced disc rupture at multiple levels including C3-4, C4-5, C5-6 and T1-2 as a result of the injury of November 27, 1999. Dr. Tator states after examining the X-rays of December 1999, that the Plaintiff had no pre-existing trauma or symptoms from the ossification of the posterior longitudinal ligament prior to the accident at Home Depot. He further states that the injury of November 27, 1999 is responsible for her condition becoming symptomatic and if she had not sustained the said injury, it is very likely that this condition would not have become symptomatic. He adds that the injury resulted in spinal cord compression in her neck and a myelopathy and that in fact, this myelopathy will be permanent.

[31] Dr. Tator says that the effects include slight weakness of the left upper limbs and slight sensory symptoms in the left side of her body. According to him, she sustained a slight concussion in the injury of November 27th and continues to have some post-concussive

symptoms, especially headaches. The doctor testified that it is not necessary to lose consciousness to sustain a concussion. He found that her condition got worse over time. She noticed numbness in her left hand. He said that this diagnosis encompasses an injury to the nerves or spinal cord or both.

[32] Dr. Tator also says that the Plaintiff's headaches have some features of migraine and since she did not have these prior to her injury, the injury to her head appears to have provoked the development and persistence of headaches which have some migraine features.

[33] Dr. Tator concludes by saying that the Plaintiff would have to be followed in the future with periodic MRI's to ensure that she does not develop any further spinal compression and she should also have a CT scan from C2 to T1. He adds that monitoring will be required for the rest of her life in order to provide early detection of any further development of spinal cord compression. Dr. Tator saw the Plaintiff again on June 21, 2006 and he concluded that her condition remained relatively unchanged.

[34] The Defendant's expert witness, Dr. Taylor, is of the opinion that the only evidence of a neurological deficit is contained in Dr. Bélanger's preoperative evaluation in 2003. Dr. Tator rejects that position based on the chiropractor's 2001-2002 notes and the physiotherapist's notes taken within days of the 1999 incident. He believes that the Plaintiff sustained neurological injuries based on the December 10, 1999 notes by Janet Sprague, the Plaintiff's physiotherapist. The notes described the Plaintiff's back, neck and shoulder pain, as well as the Plaintiff's reports of pins and needles in her left hand. In Dr. Tator's opinion, pins and needles only occur where

there is some injury to the nervous system, but not where there is injury to ligaments, bones or muscles.

[35] Dr. Tator believes the notes from Dr. Jackson, the Plaintiff's chiropractor between November 2001 and January 2002, clearly document manifestations of neurological injuries sustained 1999. In particular, Dr. Tator notes, the loss of motion in the Plaintiff's neck, the loss of her grip strength, and the marked reduction of her pinched strength. Dr. Tator believes that the Plaintiff's report of cold sensations on the left side of her body are a significant indication of a neurological injury.

[36] Dealing with the 2003 MRI and the 1999 X-ray, Dr. Tator agrees with Dr. Bélanger, the surgeon who operated the Plaintiff, that the 2003 MRI is consistent with a mild spinal cord injury.

[37] In Dr. Tator's opinion, the effect of the ossification of the posterior longitudinal ligament (OPLL) was to create a bulging disc that compress the nervous tissue in the spine, thereby leading to the Plaintiff's symptoms of weakness in her left hand and numbness over the left side of her body.

[38] Dr. Tator concludes that if the OPLL had been prominently present in 1999, then the December 2, 1999 X-ray would have revealed it. Even though the OPLL was not evident in that X-ray, he believes OPLL was still likely present to some extent at the time of the Plaintiff's injury in 1999.

[39] It is Dr. Tator's opinion that the Plaintiff's pre-accident history did not contribute to the Plaintiff's ultimate condition for two reasons. First, the 1997-1998 symptoms in the Plaintiff's neck as well as the infections in her upper respiratory tract and lymph nodes, did not indicate a pre-existing condition. Second, he did not believe that the minor injury the Plaintiff received at Loblaw's grocery store ten days before the Home Depot accident inflicted any neurological damage.

[40] Dealing with the relationship between the 1999 trauma and the Plaintiff's OPLL, Dr. Tator states that trauma can accelerate degenerative disc disease and cause acute disc ruptures. In his view, it is very likely that the Plaintiff's discs were broken as a result of the 1999 trauma.

[41] In Dr. Tator's view, the Plaintiff ultimately required the 2003 surgery to alleviate compression of the spinal cord based on two factors. One, the trauma of her disc accelerated the generative disc disease and two, the OPLL itself. Dr. Tator states that for the 1999 trauma, he would not have expected the Plaintiff to have suffered progressive neurological deficits requiring surgery in 2003.

[42] Dr. Tator rejects Dr. Taylor's position that an infectuous process may have driven the OPLL. Dr. Tator's experience with hundreds of OPLL patients leads him to believe that infection is not an identified cause of this condition.

[43] Dr. Taylor says that in terms of continuous symptoms after the surgery performed by Dr. Bélanger, the Plaintiff could still expect to suffer from disrelated pain. Also, he says, the surgery had the unfortunate affect of causing the loss of additional mobility.

[44] Dr. Tator rejects thoracic outlet syndrome as the cause of the Plaintiff's current pain. He saw no evidence that the Plaintiff's cervical ribs were accounting for any of the Plaintiff's systems either before or after the 1999 trauma. It is his opinion that there is no need to postulate the presence of other syndromes when there is definite compression of the spinal cord. Instead Dr. Tator believes that the soft tissue injuries that the Plaintiff received when the box hit her can lead to chronic myofascial pain.

[45] Dr. Tator was cross-examined on the fact that his report mistakenly referred to the box falling on the Plaintiff, while the Plaintiff testified that an employee struck her with the box. In response, Dr. Tator stated that while the acceleration of the box was an important factor, it would not have altered the symptoms the Plaintiff suffered as a result of the event. The Home Depot incident caused the Plaintiff's disc to rupture and accelerated her OPLL in Dr. Tator's opinion.

[46] In cross-examination Dr. Tator conceded that it is unusual that a multi disc rupture was not discovered until 2003 nearly three and a half years after the Home Depot incident.

[47] Again in cross examination Dr. Tator rejected the Defendant's thoracic outlet syndrome theory. Although he conceded that the Plaintiff's constellation of symptoms could indicate thoracic outlet syndrome, he concludes that it would be an unlikely diagnosis especially because there is no leg symptom. Dr. Tator maintained that a virus, possibly mononucleosis produced the

Plaintiff's enlarged lymph nodes and neurological symptoms in 1997-98 as opposed to thoracic outlet syndrome. Dr. Tator said in his opinion, based on personal experience, trauma can worsen with OPLL however he concedes that he cannot provide definite scientific evidence that trauma can aggravate or accelerate OPLL.

[48] Dr. Stotts says that when he last saw the Plaintiff on July 19, 2006, she was having no substantial relief of her chronic neck pain and headaches which he felt was secondary to her neck pain. He also said that it appeared that her pain originated in 1999 when she was struck on the back of the neck while shopping at Home Depot. Dealing with her future prognosis, the doctor says that given her pain has continued for seven years, her prognosis is poor for a complete pain free recovery.

[49] Dr. Stotts referred the Plaintiff to Dr. Alex McKee who also testified at the trial. Dr. McKee is a specialist in physical medicine and rehabilitation. He first examined the Plaintiff on January 31, 2006 and found that she had a significantly restricted range of motion of the cervical spine particularly into extension. He also found marked tenderness over the Plaintiff's left greater occipital nerve with moderate tenderness over the left C1-2 region and marked tenderness over the left C2-3 and C3-4 facet joints with moderate tenderness at C4-5 and C5-6 and on the left, from T3 to T7. He recommended that the Plaintiff be treated by a massage therapist which she did for intra-muscular stimulation but she found it extremely painful and could not tolerate the treatment. She tried going on a treadmill but noted a lot of squeezing pain in her head after the exercise. The Plaintiff took physiotherapy at the Bank Professional Physiotherapist in September, October and November 2006. When Dr. McKee saw her in

September 2006, he encouraged her to go for the next four to six weeks to take physiotherapy. Dr. McKee also found that it was well documented that the Plaintiff had pain and myofascial tenderness in the neck since the accident. It is his opinion that there is no way of diagnosing this without examining the patient. Dr. McKee said that before November 1999, he saw nothing as a contributor to her condition today.

[50] The Solicitor for the defense asked Dr. G. M. Taylor to prepare a medical opinion with respect to the Plaintiff based on a review of the medical information supplied to him. Dr. Taylor is a retired orthopedic surgeon with experience in spinal surgery. He is also retired from his position as a professor at the Queen's University school of medicine. He is presently a consultant with the Workers' Safety and Insurance Board and travels north and on the west coast of the James Bay four weeks per year to do orthopedic medicine in the communities up the coast. He is also actively involved in medical litigations such as the case at bar.

[51] Dr. Taylor does not think the 1999 trauma at Home Depot is the cause of the Plaintiff's condition. In effect, he says that the 1999 trauma and the OPLL are coincidental and not causally related. He rejects Dr. McKee, Dr. Stotts and Dr. Tator's opinion that the 1999 incident at the Home Depot caused multi-level disc herniation at C3-4, C4-5, C5-6 and in the thoracic level. Instead he believes that the Plaintiff sustained a muscular ligamentous injury to her neck in 1999 which could not be the cause of her present condition.

[52] The Plaintiff's doctors believe the 1999 trauma aggravated or accelerated her OPLL.

[53] Dr. Taylor presents an alternative explanation. He believes that there is something systemic in the Plaintiff's spine that would have happened regardless of the 1999 trauma. His conclusion is based on his experience that an isolated trauma to the neck does not result in degeneration to the multiple discs in the spine. In his experience, he said a neck injury that is muscular ligamentous in nature does not develop in OPLL at multiple levels.

[54] Dr. Taylor says that while the 1999 trauma occurred at an isolated level of the Plaintiff's neck, the Plaintiff suffered from OPLL at multiple levels of her spine according to the 2003 MRI. As a result, Dr. Taylor believes that the majority of the Plaintiff's disabilities are due to a preexisting degenerative disease that caused the OPLL and not specifically to her injury.

[55] Dr. Taylor says that the evolving degenerate change was not evident in the X-rays of 1999. However, he believes that the evolution of the degenerate change is evidenced by the Plaintiff's symptoms in late 2001 and 2002. He is unable to say what the cause of the preexisting degenerative disease with any certainty.

[56] Dr. Taylor also believes that the Plaintiff suffers thoracic outlet syndrome due to a cervical rib, and unusual anatomical feature. Thoracic outlet syndrome he says is indicated by the symptoms the Plaintiff complained of in 1997 and 1998. In his opinion, the thoracic outlet syndrome accounts for some symptoms suffered by the Plaintiff after her 2003 surgery.

[57] Dr. Taylor also says that there is no indication of herniated disc following the 1999 incident in the notes taken by the Plaintiff's medical practitioners and he believes that the need for the 2003 surgery was caused by a bony overgrowth i.e. OPLL rather than a herniated disc.

[58] Dealing with the role of thoracic outlet syndrome before and after the 2003 surgery, it is Dr. Taylor's opinion that the said thoracic outlet syndrome played a part in the Plaintiff's condition both before and after the 2003 surgery by Dr. Bélanger. He believes that the Plaintiff's thoracic outlet syndrome is caused by a cervical rib, which lessens the volume of space in the triangle between the *scalenus anticus* and *medius* in the neck. The result, he says, is that the nerves that exit the neck under the clavicle are affected by the reduced volume of space.

[59] Dr. Taylor believes that thoracic outlet syndrome affects the Plaintiff's clinical picture as of time of trial. He believes that the history of her progressive disability, progressive pain, progressive limited motion, progressive disability is creating a spasm, tightness of her *scalenus anticus* and *medius* in response to her pain which is aggravated by stressful situations which would be aggravated by lifting. He says that all these things would make the thoracic outlet space smaller and would be provocative to the symptoms she would be experiencing in her left arm that is the tingling of her fingers and tenderness on her forearm and the pain.

[60] Dr. Taylor notes several indications of thoracic outlet syndrome in the Plaintiff prior to the 1999 trauma. First, he says, the Plaintiff complained of numb left arm as far back as 1997. According to him, the only way the Plaintiff could have numb left arm is if all five nerve roots C5, C6, C7, C8 and possibly T1 are interfered with as they exit the spinal column. That would be unlikely in Dr. Taylor's opinion. So Dr. Taylor concluded that the only alternative that could cause the Plaintiff's arm to go numb in five different spots is that the nerves are all affected in the place where they are all together, in the triangle in the neck. The irritation of those five nerves then causes pain in the Plaintiff's arm before and after surgery, he says.

[61] Dr. Taylor also rejected Dr. Stotts' opinion that the Plaintiff would have a neurological deficit in her left thumb if she had thoracic outlet syndrome. Dr. Taylor notes that in 1997-1998, the Plaintiff reported occasional intermittent numbness of her three middle fingers, a sharp pain on the left side of her neck with radiation of the left shoulder and left arm, and a reduced range of motion in her neck.

[62] Dr. Taylor disagrees with Dr. Tator, Dr. Stotts and Dr. McKee who all concluded that a viral illness, mononucleosis, was responsible for the 1997-1998 symptoms. Dr. Taylor therefore said instead he believes that the Plaintiff's 1997-1998 symptoms can be explained by thoracic outlet syndrome.

[63] Dr. Taylor rejects the notion that because the Plaintiff has myofascial pain immediately following the 1999 trauma, that the trauma is therefore the cause of her current myofascial pain. He states that myofascial pain only identifies where pain originates from (i.e. muscle and fibrous tissue). He states that the Plaintiff's current myofascial pain may be caused by the scarring from surgery, the ossification which affects her range of movement and the irritation of the thoracic outlet, none of which were caused by the 1999 trauma at Home Depot, in his opinion.

[64] In cross-examination, Dr. Taylor concedes that he does not know what caused the Plaintiff's OPLL. He wonders whether the OPLL might be caused by an infectious process but cannot say with any certainty. He bases his theory on the low grade fever, fatigue and neck pain suffered by the Plaintiff in 1997-1998.

[65] Dr. Taylor's opinion is based on the notes of other doctors and he concedes that some doctors take very good notes while others take poor ones. Dr. Taylor believes that thoracic outlet syndrome can be diagnosed without seeing the patient despite the fact that Dr. Stotts says that such a diagnosis is impossible. Dr. Taylor notes that he has seen thousands of cervical spine X-rays and has never seen OPLL as incidental finding in somebody who has an acute injury however he concedes that his conclusions are based on two recent cases that he has seen and the small body of literature on the subject.

[66] Dr. Taylor does not think that it is possible to know with any certainty why a disc degenerates.

[67] There was a significant concession made by Dr. Taylor in response to a question as to whether he could see any connection between the trauma of 1999 and the OPLL in 2003. His answer was as follows "Oh – there – there may be. It may have contributed to but I don't believe that – that it's the sole reason for her...neck – radiological changes." However Dr. Taylor continues and says "I find it hard to believe....that this lady could have that amount of change as a consequence of a blow on the back of her head and neck with a 26 lb box" but he concedes that he cannot say whether the Plaintiff's cervical ribs or her thoracic outlet syndrome materially contributed to her symptoms. He can only say that the Plaintiff's cervical ribs and thoracic outlet syndrome probably contributed to her current condition.

[68] Dr. Stotts rejects the Defendant's thoracic outlet syndrome theory because the Plaintiff's symptoms are not in keeping with the syndrome in the following respects:

- The Plaintiff's numbness was throughout the left arm, as opposed to thoracic outlet syndrome which is quite particular.
- The thoracic outlet syndrome does not usually generate the high level of pain experienced by the Plaintiff.
- The cervical ribs do not always lead to thoracic outlet syndrome, as indicated by the presence of cervical ribs in 33% of the population.
- There was no evidence that an artery, vein, or nerve structures of the Plaintiff were compressed.

[69] In Dr. Stotts' own words "the possibility of thoracic outlet syndrome just won't have entered into his consideration as a possibility". Dr. Stotts is confident that the Plaintiff's pain following her surgery is myofascial in nature. Myofascial pain pertains predominantly to connective tissues such as muscles, tendons and ligaments. Dr. Stotts was sure of this conclusion because of as March 4, 2004, he had excluded any neurological damage including substantial spinal cord compression, nerve, or brain damage. His conclusions are not changed by the fact that the Plaintiff was treated for a virus in 1997-1998.

[70] In cross-examination, Dr. Stotts rejects the notion that the 1999 X-ray would have picked up a small disc herniation because X-rays are not sophisticated enough to pick up small herniations or small changes in alignment.

[71] Dr. Stotts concedes that the Plaintiff exhibited potential symptoms of thoracic outlet syndrome but he says he rejected thoracic syndrome because there was a more predominant reason for the symptoms and he did not have any clinical findings of the syndrome.

[72] Dr. Stotts concedes that he cannot put a date on when the Plaintiff became sensitized to pain. He concedes sensitization may have risen between late 2001 and between the first time the Plaintiff saw him in March of 2004.

[73] At the request of the defense, the Plaintiff was examined by Dr. Wayne Quan, a psychiatrist. Dr. Quan received his fellowship in 1976 and has been practicing psychiatry in Ottawa since that time. He is a professor at the school of medicine at the University of Ottawa and the editor of the Psychiatric Journal of the University of Ottawa.

[74] On September 14, 2005, Dr. Quan examined the Plaintiff. He said that his examination lasted between one and a half and two hours. Prior to the examination, he had the opportunity of reviewing a number of medical reports and notes that had been prepared by several doctors consulted by the Plaintiff in the past going back to 1987. He says that the documentation provided indicated that the Plaintiff had been experiencing preexisting symptoms of headaches, fatigue and left shoulder pains well before the accident of November 27, 1999 at Home Depot. He also refers to the accident at Loblaws grocery store which occurred before the accident at Home Depot and he says that the Plaintiff never mentioned it to him and in fact she denied any other accident other than the one at Home Depot.

[75] He states that Dr. Lentz' report indicated that she had been experiencing long standing marital tension and that her perception of pain increased with increase marital problems. In his report prepared on October 18, 2005 and filed with the court, Dr. Quan states that on the basis of his assessment, and on the basis of the history provided to him, it was very evident that the Plaintiff did satisfy the criteria for a diagnosis of pain disorder associated with both

psychological factors and a general medical condition. Her perception of pain, he says, has caused significant distress and impairments of her occupational and other important areas of functioning and has resulted in the frequent use of the health care system and pain has become a major focus of her life. He adds as frequently as the case is the present symptoms often accompany chronic pain. In the case of the Plaintiff, it is his opinion that the Plaintiff did manifest evidence of mild degrees of depressive symptomatology.

[76] Mrs. St.Prix-Alexander has cervical ribs according to Dr. Quan. He says that it would seem that she had been experiencing pain in the neck and shoulder area for a number of years before the accident in the Home Depot store. In fact, he says, she reported symptoms consistent with the diagnosis of thoracic outlet compression syndrome. The doctor then says “while you do need the opinion of a specialist in the area, it would seem that the accident at Home Depot store had very little to do with the Plaintiff’s pain condition”. And then he says “and had been in existence before the accident and the accident may have merely exacerbated the pre-existing condition or given her an attribution for pains experienced”. In any event, the doctor says that the Plaintiff is contending with a chronic pain condition and that psychological factors do adversely influence her perception of pain. In addition to chronic pain, he says the Plaintiff is subject to classic migraine headaches which are currently being treated by Dr. Stotts.

[77] Five doctors testified at the trial of this action, they are Dr. Stotts, a neurologist, Dr. McKee, a physiatrist and Dr. Tator, a neurologist who testified on behalf of the Plaintiff and Dr. Quan, a psychiatrist and Dr. Taylor, an orthopedic surgeon who testified on behalf of the Defendant.

[78] The Plaintiff saw Dr. Stotts eleven times, Dr. McKee, five times and Dr. Tator examined her on two occasions. Dr. Stotts believes that the 1999 trauma is responsible for the symptoms the Plaintiff continued to suffer after the 2003 surgery and rejects the thoracic outlet syndrome. Dr. McKee found that the Plaintiff continues to have significant symptoms as a direct result of the injury she received at Home Depot in November 1999. Dr. Tator, the Plaintiff's expert witness, is of the opinion that the Plaintiff had a major injury to her cervical spine which produced disc rupture at multiple levels including C3-4, C4-5, C5-6 and T1-2 as a result of a major injury on November 27, 1999. Dr. Tator said that the Plaintiff had no preexisting trauma or symptoms from the preexisting of the ossification of the posterior longitudinal ligament prior to the trauma. He says that the injury is responsible for her condition becoming symptomatic and if she had not sustained the injury, it is very likely that this condition would not have become symptomatic. Dr. Tator also says that the injury resulted in a spinal cord compression in her neck and a mild myelopathy, and the effects of this myelopathy will be permanent. He explained the effects as including slight weakness of the left upper limb and slight sensory symptoms in the left side of her body. He also says that the Plaintiff sustained a concussion in the injury of November 27, 1999 and continues to have some post-concussive symptoms especially headaches. Her headaches have some features of migraine but she did not have these prior to the injury and thus the injury to her head appears to have provoked the development and persistence of headaches which have some migraine features. These, he says, have been psychological effects of the injury. Dr. Tator then concludes by saying that she will have to be followed in the future with periodic MRI's to ensure that she does not develop any further spinal cord compression. He says she should have a CT scan from C2 to T2 and that monitoring will be

required for the rest of her life in order to provide early detection of any further development of spinal cord compression.

[79] Dr. Taylor believes that the ossification of the posterior longitudinal ligament (OPLL) and the 1999 trauma at Home Depot are not causally related. In fact, he believes that they are coincidental. His opinion is that the 1999 trauma only inflicted a muscular liguamentous injury and not a multi-disc herniation. Dr. Taylor's theory is that the Plaintiff's neurological type symptoms both before and after the 2003 surgery are attributable to thoracic outlet syndrome which would be unrelated to the 1999 trauma. He therefore concludes that the 1999 incident had very little to do with the problems that the Plaintiff suffered throughout until today.

[80] Dr. Quan also believes that the accident at Home Depot had very little to do with the Plaintiff's pain condition. He states that the Plaintiff is contending with a chronic pain condition and that psychological factors adversely influence her perceptions of pain and she may benefit from treatment in a chronic pain clinic. He also concludes that the pain she has been experiencing is consistent with the diagnosis of thoracic outlet syndrome.

[81] I agree with the conclusion reached by the expert witness Dr. Tator that the Plaintiff sustained a major injury in her cervical spine as a result of the accident at Home Depot on November 27, 1999 and that the said injury responsible for her condition becoming symptomatic. I also agree with his opinion that the injury resulted in spinal cord compression in her neck and a myelopathy and the effects of this myelopathy will be permanent. The doctor says that the effects include slight weakness of the left upper limb and slight sensory symptoms in the left side of her body. I also accept Dr. Tator's evidence that the Plaintiff sustained a concussion

in the injury of November 27th and continues to have some post-concussive symptoms especially headaches which have some features of migraine which she did not have prior to the injury and that the injury to her head appears to have provoked the development and persistence of headaches which have some migraine features.

[82] Dr. Stotts concluded that the 1999 accident was causative of the headaches he was treating the Plaintiff for as she did not have frequent substantial long-lasting headaches before the said accident. He concludes that it is unlikely that the Plaintiff's chronic pain will substantially diminish given its lengthy history.

[83] I favour the opinion given by Dr. Tator over Dr. Taylor because Dr. Tator examined the Plaintiff on two occasions while Dr. Taylor never set eyes on the Plaintiff. Dr. Taylor's opinion and testimony is based on a review of medical information supplied to him by the Defendant's lawyer. No explanation was given to the Court of the reasons, if any, why Dr. Taylor did not examine the Plaintiff. I agree with Dr. Tator and Dr. McKee that some diagnosis cannot be made without examining the patient. Some medical conditions can only be determined by such examination.

[84] At the time of the accident, the Plaintiff was on parental leave having given birth to twins in September of 1999. She said that she intended to go back to work in April of 2000 but extended her unpaid leave until September 2000.

[85] She returned to work as a public servant in August 2000 as the Executive Assistant to Marcel Nouvet, an associate Deputy Minister who was the Chief of the Human Resources office

at Treasury Board of Canada. Mr. Nouvet testified at the trial. He said the Plaintiff worked as his Executive Assistant from August 2000 to February 2002 and met more than his expectations. He told us that she is very intelligent, has excellent communication skills in both official languages, is committed, dedicated, loyal, worked long hours and is a real dynamo. In a letter addressed to the Plaintiff in June 2001, he informed her that because of her outstanding performance, she would receive an increase in salary and a bonus. At the end of the letter, he wrote a PS as follows “The future is yours. Merci pour tout.” When asked at the trial why he wrote that, he said that she had great potential, she had everything going for her, she was intelligent, worked beyond the call of duty, she was dedicated, skilled, had experience outside the public sector and was a member of a visible minority.

[86] Mr. Nouvet said the Plaintiff complained of headaches at times during the 18 months she assisted him but she kept working 60 or more hours per week. In February 2002, he said she moved to Health Canada where she worked until March 2006. After she moved, Mr. Nouvet said he did not see her as often but he remembered that after her operation of 2003 she did not look well and he wondered why she came back to work so soon instead of looking after herself. After that he said that as far as her capacity to work was concerned, he noticed that she was losing ground all the time. He said that as time went by between February 2002 and 2006, he saw her about twice per year and saw that her illness was catching up to her. She looked old, tired and missed several days of work.

[87] Between February 2002 and March 2006, the Plaintiff was employed as the Executive Director of the Women’s Health Bureau at Health Canada. She had to take an

extended leave of absence between the months of April and September 2003 for the surgery performed by Dr. Bélanger. With the Health Bureau, she found that she had a little more control over her work and was expected to do less traveling and meetings. Nevertheless, during that period of time, the Plaintiff says that she was exhausted when she got home and her activities were very limited. Until the month of March 2006, she was taking both Wednesdays and Fridays off and made up for it at other times. In 2006 she changed boss and he required her to be present at the office and wanted weekly meetings and reports. She said that she could no longer be accommodated as she was before and she felt that she had to quit her job because it was not physically possible for her to stay on.

[88] The Plaintiff then commenced working as Director of Human Resources Policy and Programs at Natural Resources Canada where she worked commencing in March 2006. Natural Resources was a smaller department and before taking the job there, she had made it known that she needed one day off per week on Wednesdays or Fridays. At first, this was going well but after one month, she had a new boss who was not as flexible as the previous one and expected her to be there every day. In June 2006, she left that employment and applied for long-term disability.

[89] The Plaintiff says that she still suffers from very bad headaches and that she is having more “bad days” now than two years ago. She also still complains of numbness and tingling sensations. She is still under Dr. McKee’s care and last saw him one month before the trial. He is prescribing more physiotherapy and massage therapy. The Plaintiff sees no improvements in her condition and says that joint injection will be done to her at the hospital in the future.

[90] The Plaintiff says that she does not expect that she will be able to return to work because she is not able to function properly. She does not feel that she could even do part-time work. She said she had planned to work as long as she could and at least until she was 65 years of age and intended to do some consulting work after that time.

[91] When dealing with the Plaintiff's employability, it is Dr. Tator's opinion, that chronic neck pain can be very demoralizing and can cause migraines such as those suffered by the Plaintiff. He believes the Plaintiff's ability to work is significantly compromised namely by the pain issue. He states that in an executive type stream that the Plaintiff has been in for her career, it's extremely difficult for people with chronic pain to be involved in that type of work.

[92] When Dr. Stotts was asked if the Plaintiff would be able to return to work in the future, he said that she does not have a specific neurological deficit to prevent her from going back to work however she is substantially limited by chronic pain and she has developed a reactionary depression to this pain. He concludes by saying that he cannot predict whether or not the symptoms will be relieved to a degree where she can return to work but it is his neurological opinion that if chronic pain has existed for this length of time that it will be difficult to substantially diminish.

[93] When asked about the Plaintiff's capacities to work Dr. McKee said that it had not been a focus of his involvement with the Plaintiff. He concludes by saying that he cannot comment at this time whether at some point in the future, she will be able to work part-time at a lesser position.

[94] The Plaintiff was tested by Ross Skirda, an employability examiner. Mr. Skirda prepared a report and testified at the trial. He was asked to specifically examine how the Plaintiff's injuries may have affected her forward career movement and to what extent. In order to achieve his work, the Plaintiff made three attendances at his office. On her first attendance Mr. Skirda said that within a few hours of being with the Plaintiff, he found her to be unable to carry on a conversation and unable to express herself in an effective manner. During the second day of testing within a shorter period of time than the first day, the Plaintiff reported a complete inability to concentrate. I find it strange that the Plaintiff did not display similar difficulties when testifying in this court for more than 13 hours at the trial. In fact, I found that she had a very good memory and was able to remember details of illnesses and jobs going as far back as her college days. She had no difficulty whatsoever testifying in a very effective manner.

[95] The Plaintiff's position now almost seven and half years after the accident, is that she will not be able to work in the future and she is claiming \$4,000,000 for loss of working capacity.

[96] At the time of the accident at Home Depot in November 1999, the Plaintiff was scheduled to return to work following her leave to the private sector on November 2, 1999. With the birth of her twins in September 1999, she extended this leave until April 30, 2000 and then extended it again to August 2000. As I said, she told us at trial that the reason for the extension was because of the pain she had as a result of the injuries sustained in the accident at Home Depot but she told her employer that it was for care and nurturing. When asked why she was not truthful with her employer, she said that she had no obligation to tell her employer the real reason she wanted additional time off and she did not have to tell her employer that she was

ill unless asked directly. She was therefore not honest with her employer thinking that it would be to her advantage in the workplace to give an untruthful reason for extending her leave.

[97] During the month of August 2000, she returned to work as the Executive Assistant to Mr. Bouvet working an average of 60 hours per week until February 2002. Between February 2002 and March 2006, she worked for a while as Executive Director of the Women's Health Bureau, a department of Health Canada, and in her last months, she worked for Resources Canada and then went on long-term disability during the month of June 2006.

[98] I agree with the solicitor for the defense who said in his argument that it seems that at every opportunity, the plaintiff exaggerated her medical condition. I find that on several occasions, she was not entirely truthful in her answers.

[99] Because of several reasons, I find that the Plaintiff's evidence is not credible. For instance, she could not recall whether she hurt her right arm or left arm when she fell at the Loblaws grocery store 10 days before the accident at Home Depot but could recall a cold or flu she had in the mid-1980's when she was a student at university. Also, she testified that the retirement allowance she received at Levi Strauss plant when it closed down was approximately \$50,000 when in fact it was \$104,000. Another example is that according to the Plaintiff, any prior problems with her neck or neurological symptoms were all related to a viral infection she had. She insisted that the doctor at the walk-in clinic on the date of the Home Depot accident told her that she had lost consciousness when no such diagnosis was written in the doctor's clinical notes.

[100] The Plaintiff said that since the accident at Home Depot, she has experienced constant pain and headaches. Dr. Morris, her family doctor, referred to the Plaintiff having pain in the head and neck on only two occasions, on December 2 and 16, 1999. Although he has examined her at least on 40 occasions since the accident at Home Depot, he mentioned pain only twice in his notes. The Plaintiff said she complained at almost all the visits she had with him regarding the pain. I find it difficult to believe that Dr. Morris was told about the pain and headaches at almost every visit and referred to this only twice in his notes. If that is so, Dr. Morris should have been called as a witness to tell us that his notes were inaccurate on that subject. Dr. Morris should have been called as a witness at this trial. He would have been of great help to the court. All this court had from Dr. Morris was his hand written clinical notes. They were difficult to read and difficult to decipher. No medical report was filed by Dr. Morris and no explanation was given to the court why he was not called as a witness or he was not asked to prepare a full report. In fact, Dr. Morris is the only doctor who treated the Plaintiff regularly before and after the accident at Home Depot and is still the Plaintiff's family doctor. Dr. Morris would have been of more help to this court than the several doctors who testified and examined the Plaintiff four, five and six years after the accident.

[101] I find that there is not sufficient evidence to permit me to award loss of income damages to the Plaintiff from the time she commenced receiving long-term disability in June 2000 until she attains the age of retirement. The Plaintiff is only of 42 years of age. Except for the time she was on maternity leave and the leave of absence that she took when she was operated in 2003, she worked mostly on a full-time basis until the month of June 2006. For years after the accident, she was working 60 hours plus per week at a very stressful job.

[102] I have therefore come to the conclusion that the Plaintiff could have continued working as a public servant. No effort was made by her to find a less stressful job or to find out if she could be accommodated by her employer by working regular hours at a job with less responsibilities.

[103] It is clear that the Plaintiff was injured in the accident at Home Depot and that the injury sustained by her caused her a lot of inconvenience and suffering however, I do not believe that she will be unable to work in the future. Her doctor did not recommend that she cease working. It was a decision she made on her own and the doctors were faced with a “*fait accompli*” because she felt that she could not go back to work. She is now on long-term disability and receiving 70% of her salary but there was no evidence introduced at the trial to indicate the medical reason why she is on long-term disability. I have no doubt she has a loss of competitive advantage because of the injuries sustained by her and she will suffer economic loss because of the fact that she can no longer work 60 hours per week and her ability to compete for employment has been impaired. She can no longer accept jobs that demand a lot of concentration and as stressful as an Executive Assistant. She will therefore be less attractive to her employer than a comparable individual not impaired in any way. The fact that she is now unable to work at the Executive 1 or 2 levels in the Public Service is compensable in damages.

[104] Dealing with the Plaintiff’s claim for past loss income, she is claiming \$29,093 for a five month period from April to September 2000 when she extended her unpaid leave. I find that the leave was extended for family related responsibilities and that the accident of November 27, 1999 had nothing to do with this leave. I am therefore not allowing her claim for

this amount. With respect to the family law claims, no evidence was offered to show the impact of the Plaintiff's injury upon her three children. Some evidence was offered with respect to the assistance rendered by her husband, Michael Alexander, with the children and with the help given to the Plaintiff, especially after her operation of April 2003. I will therefore assess *Family Law Act* damages for the husband and deny the claim of the children.

[105] I will also allow OHIP's subrogated claim and the amount claimed by the Plaintiff for babysitting.

[106] In addition to compensation for loss of competitive advantage, the Plaintiff is entitled to general damages for pain and suffering.

Summary of Damages to be Paid to the Plaintiff, Deanna St.Prix-Alexander

General Damages for Pain and Suffering	\$75,000.00
Special Damages	
OHIP (subrogated claim)	\$16,162.25
Babysitting	\$6,970.00
Loss of Competitive Advantage	\$400,000.00
	<u>Total</u>
	<u>\$498,132.25</u>
Family Law Claimed Damages of Michael Alexander	\$20,000.00
	<u>Grand Total</u>
	<u>\$518,132.25</u>

[107] In the event the parties are unable to agree on interest and costs, I am to be spoken to.

Manton J.

Released: January 4, 2008

COURT FILE NO.: 01-CV-16871

DATE: 2008/01/04

ONTARIO

SUPERIOR COURT OF JUSTICE

B E T W E E N:

DEANNA ST.PRIX-ALEXANDER, MICHAEL
ALEXANDER AND SKYLER ALEXANDER,
GABRIELLE ALEXANDER and NATHANIEL
ALEXANDER by their LITIGATION
GUARDIAN, DEANNA ST.PRIX-ALEXANDER

Plaintiffs

- and -

HOME DEPOT OF CANADA INC.

Defendant

REASONS FOR JUDGMENT

Manton J.

Released: January 4, 2008